

Connectivity Assessment

Beacon + five priority 340B TPAs | Direct connectivity feasibility, dependencies and delivery path



Executive conclusion

Direct connectivity is technically practical, but it is not a “six public APIs” problem. Beacon has the clearest publicly documented external API/SDK model. Verity and Craneware publicly document scheduled SFTP delivery. PharmaForce publicly documents API-driven Beacon integration, while Macro Helix and Pillr demonstrate external connectivity but do not publish a general customer API specification. The safe architecture is therefore a hybrid connector fabric: API/SDK where proven, SFTP/EDI/structured files where that is the supported production interface, and Inmar retained only as a coexistence source until direct-feed parity is demonstrated.

Assessment perimeter

The six platforms assessed below are the same Beacon + five TPA platforms identified in the final proposal. The five TPAs are the proposal set, not a claim of external market-share ranking; build sequence should ultimately follow the actual Shields health-system footprint.

Platform	Public interface evidence	Direct API to Shields?	Recommended production path	Confidence	Working connection	Prod-ready
Beacon	API + SDK + file submission; partner read/write tokens	Yes, documented, after partner onboarding	Direct API/SDK; file fallback	High	3-5 wks	5-7 wks
Verity 340B	Scheduled encrypted SFTP data exports	Not publicly documented	Direct SFTP export	High	2-4 wks	4-6 wks
PharmaForce	API-driven Beacon submission; bidirectional Beacon reconciliation; Multi-EDI	Customer-facing API not publicly documented	Vendor API / managed export; SFTP/EDI fallback	Med-High	3-5 wks	5-7 wks
Craneware	Scheduled SFTP reports; Trisus rebate integration	External customer API not publicly documented	Direct SFTP; private API if offered	High for SFTP	2-4 wks	4-6 wks
Macro Helix	Qualified-claim reporting; certified 850/855 EDI interface	Generic customer API not publicly documented	Structured export / SFTP; private API if offered	Medium	3-5 wks	5-7 wks
Pillr / RxStrategies	Beacon supporting-entity integration; multi-source connectivity	External customer API not publicly documented	Vendor data extract/API; secure file fallback	Medium	3-5 wks	5-7 wks

Realistic program-level connector timing

Engineering build window	~8-10 calendar weeks for the six priority adapters when three integration engineers work in parallel.
Production-capable connectivity	~10-14 calendar weeks if private interface packs / access arrive in the first 2-3 weeks.
Risk scenario	~14-18 weeks if two or more TPAs require custom vendor development, contract amendments, or delayed tenant enablement.
26-week program implication	The current 26-week overall plan remains credible only if connectivity is treated as a Week 1-3 feasibility gate, not as an assumption deferred into the build.

Current regulatory context

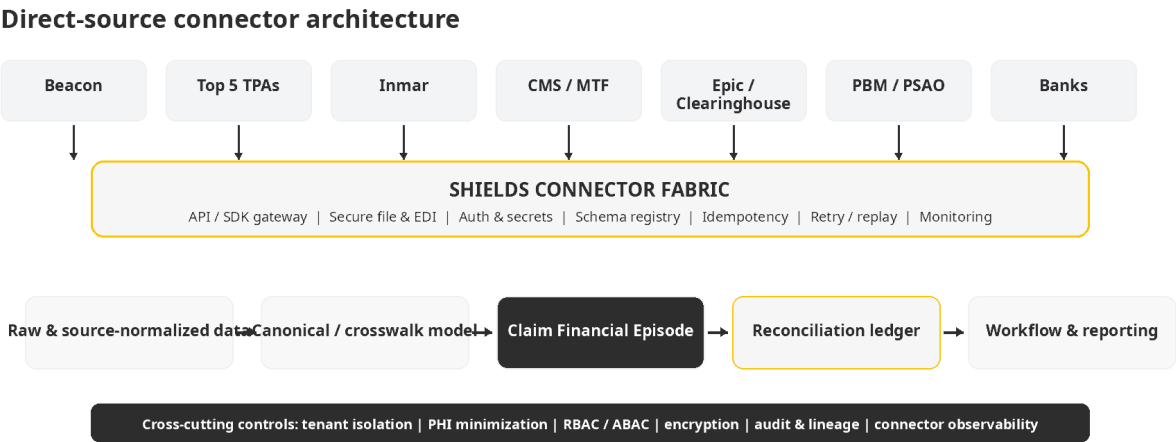
HRSA announced a revised 340B rebate-model pilot on 31 July 2026, with selected manufacturer plans expected to become effective 1 January 2027. This increases the value of front-loading Beacon / TPA readiness and avoiding a late discovery that access or data contracts are not available.

Public evidence reviewed: Final proposal scope | [HRSA revised pilot](#)

Common Connector Architecture & Technical Process

StatusNeo

How direct-source connectivity should be implemented once vendor access is confirmed



Source-of-truth boundary

TPA	Authoritative for 340B qualification / source transaction context and the TPA’s own claim status.
Beacon	Authoritative for rebate submission identifiers, validation outcomes, rebate status and Beacon-side reconciliation data.
CMS / MTF	Authoritative for MFP / refund transactions where applicable.
Bank	Authoritative for settled cash.
Shields platform	Authoritative for the consolidated claim-level expected / received / outstanding / exception financial status.
Inmar	Coexistence source during transition; not the architectural broker for direct TPA or Beacon connections.

Common six-step connector build method

- 1. **Access & entitlement:** Partner / customer access, contract/data-right confirmation, credentials, sandbox/non-prod and source support owner.
- 2. **Interface contract:** Obtain API/SDK or file/EDI specification, sample payloads, keys, status/reversal semantics, history and cadence.
- 3. **Adapter build:** Implement transport, auth, idempotency, checkpointing, schema validation, raw landing and source-normalized model.
- 4. **Business mapping:** Crosswalk 340B IDs, NDC/HCPs, claim/Rx/fill IDs, provider/site, payer/PBM, manufacturer, transaction and payment references.
- 5. **Golden-claim validation:** Trace representative paid, rejected, reversed and unmatched claims end-to-end; reconcile source counts and control totals.
- 6. **Production hardening:** Retry/replay, observability, DQ quarantine, secrets rotation, runbooks, alerting, lineage, backfill and cutover readiness.

Definition of “working connection” vs. “production-ready”

Working connection	Authorized source data reliably reaches a Shields lower environment, parses successfully, maps to the source-normalized contract, and at least one golden claim can be traced end-to-end.
Production-ready	Adds monitored scheduled operation, replay/idempotency, error handling, backfill, performance, security controls, control totals, documented support and successful reconciliation of edge cases such as reversals and partial payments.

Resource assumption behind the timeline

The estimate assumes the proposal team structure: 3 integration engineers working in parallel, supported by the US technical architect, 2 data engineers, 2 backend/reconciliation engineers, QA and platform/DevOps. Vendor waiting time is not solved by adding engineers; access/onboarding remains the key schedule variable.

Beacon

340B rebate-model platform | Primary direct-integration feasibility gate



FEASIBILITY	BUILD EFFORT	ELAPSED TO PRODUCTION	RISK CASE
High: direct API/SDK path is publicly documented	4-6 engineer-weeks	5-7 weeks	7-9 weeks if entity authorization / SDK or sandbox access is delayed

What is proven publicly

- Beacon explicitly states that covered entities and TPAs should use the Beacon SDK for direct data submissions; the older 340B ESP SDK is not valid for rebate-model submissions.
- Beacon documents a Partner model in which a covered entity can grant a partner Read or Read/Write access to data through the API and SDKs.
- Partner onboarding yields an Access Token plus a separate Private Token used to authenticate API requests.
- Beacon publishes pharmacy and medical claim data templates and supports claim-level submission history, validation outcomes and Beacon IDs.

What still requires vendor / customer confirmation

- SDK package, API reference, rate limits, versioning policy, non-production endpoint and support SLA are not exposed in full public documentation and must be obtained through Beacon Support.
- Confirm whether all required rebate/payment/status retrieval operations are available programmatically for the partner role or whether any require export/portal workflows.
- Each covered entity must be registered and explicitly authorize StatusNeo/Shields partner access; permissions may need to be repeated across applicable 340B IDs.

Recommended connector design

Recommended technical path



Direct API/SDK is publicly supported; exact endpoint package, rate limits and non-prod access are vendor-onboarding items.

Direction	Outbound eligible pharmacy / medical claims; inbound acknowledgements, validation outcomes, Beacon IDs, rebate status and reconciliation data.
Authentication	Beacon partner Access Token + Private Token; entity Admin grants Read/Write permission.
Mapping	Use Beacon's published pharmacy / medical data templates; persist Beacon ID against Shields Claim Financial Episode.
Processing	Submit final 340B-qualified claims; treat validation failures as source-data exceptions, not financial postings.
Payment close	Reconcile Beacon rebate/payment reference to bank settlement before marking cash received / claim closed.

Dependencies / acceptance conditions

Required to hold the estimate • Beacon partner onboarding completed • At least one Shields/health-system covered entity registered and ready to invite partner • Beacon SDK/API package + non-prod credentials received • Representative pharmacy and medical claim files available • Banking / 340B ID setup owned by the covered entity where payment testing is required	Fallback / design decision Use Beacon's structured file upload as a contingency for early testing / pilot if partner API/SDK access is delayed. Do not design the target state around manual portal operations. Recommendation: Make Beacon the first API/SDK connector and complete partner onboarding in Week 1; its interface pattern becomes the reference for all direct rebate interactions.
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Public evidence reviewed: [Beacon FAQ / SDK](#) | [Partner tokens](#) | [Partner permissions](#) | [Pharmacy template](#)

Verity Solutions: Verity 340B

Priority TPA | Strongest publicly proven direct extraction path via SFTP



FEASIBILITY High: using SFTP; API availability unproven publicly	BUILD EFFORT 2-3 engineer-weeks	ELAPSED TO PRODUCTION 4-6 weeks	RISK CASE 6-8 weeks if custom export fields or vendor configuration are required
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What is proven publicly

- Verity publicly documents Secure Data Exports that automatically send encrypted data through Verity SFTP with daily, weekly or monthly delivery options.
- Published export datasets include accumulations, contract-pharmacy claims backing, invoices, matches/split transactions, claims backing and unmatched claims.
- Verity describes its platform as integrated with major wholesale vendor order-entry and covered-entity information systems.

What still requires vendor / customer confirmation

- No public developer documentation was found for a general customer REST API; do not assume API access in the estimate.
- Confirm which SFTP export products are licensed for each Shields-associated covered entity and whether fields include all matching keys required for the reconciliation model.
- Confirm incremental-vs-full-file behavior, historical backfill availability, file versioning and how reversals/requalifications are represented.

Recommended connector design

Recommended technical path



SFTP delivery is publicly documented; do not base the estimate on a REST API unless Verity supplies a private API specification.

Direction	Primarily inbound to Shields: qualified claim / dispense context, accumulations, matching results, invoices and unmatched transactions.
Transport	Verity SFTP secure exports: use direct file landing, checksum, file-level idempotency and source-control totals.
Mapping	Map 340B ID, site / contract pharmacy, Rx/claim identifiers, NDC, qualification/match status, invoice and source transaction keys.
Cadence	Recommend daily export for financial operations; retain vendor file name, generated timestamp and export type for audit lineage.
Beacon relationship	Use Verity as qualification/source context; Beacon submission/status can remain a separate Shields direct connector.

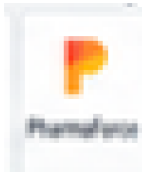
Dependencies / acceptance conditions

Required to hold the estimate • Customer approval to enable Secure Data Exports • SFTP account / key exchange and network allowlisting • Export dictionary + representative files • Daily cadence enabled • Agreement on backfill and correction/reversal semantics	Fallback / design decision If SFTP enablement is delayed, consume controlled manual exports for mapping and golden-claim validation while the production feed is being provisioned. Recommendation: Treat SFTP as the committed baseline. API should be an optimization only if Verity provides a private supported specification.
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Public evidence reviewed: [Secure Data Exports](#) | [Verity 340B hospitals](#)

PharmaForce

Priority TPA | Strong public evidence of API-driven Beacon integration, but customer API exposure needs confirmation



FEASIBILITY Medium-High	BUILD EFFORT 3-5 engineer-weeks	ELAPSED TO PRODUCTION 5-7 weeks	RISK CASE 7-9 weeks if no customer API/export exists and a custom vendor feed must be built
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What is proven publicly

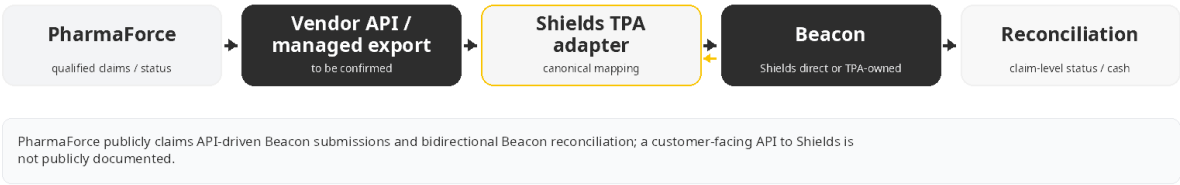
- PharmaForce ClaimsConnect is positioned as direct manufacturer reporting infrastructure through ESP and Beacon, with claim-level data and rebate-lifecycle visibility.
- PharmaForce publicly states that its rebate solution uses API-driven submissions and supports claim-level, bidirectional Beacon reconciliation.
- Its Split Billing product publicly describes Multi-EDI support and complete claim / configuration visibility.

What still requires vendor / customer confirmation

- The public material proves PharmaForce-to-Beacon integration capability; it does not prove that PharmaForce exposes a customer-facing API that Shields can independently call.
- Obtain the customer / partner interface specification: endpoints or managed export, authentication, data contract, history, cadence and SLA.
- Confirm submission ownership to prevent duplicate Beacon submissions: Shields direct-to-Beacon vs. PharmaForce-managed submission must be explicitly decided per covered entity.

Recommended connector design

Recommended technical path



Direction	Inbound TPA-qualified claims, qualification / routing result and rebate-relevant source data; optional inbound status if PharmaForce continues to own Beacon submission.
Preferred transport	Vendor-supported API or managed machine-readable export if available under customer entitlement.
Fallback	Secure structured file / SFTP or EDI data extract; map into the same canonical TPA event contract.
Beacon mode A	Preferred target: PharmaForce provides qualification; Shields submits to Beacon and independently consumes Beacon status.
Beacon mode B	Coexistence: PharmaForce submits to Beacon; Shields consumes TPA + Beacon outcomes for independent reconciliation.

Dependencies / acceptance conditions

Required to hold the estimate - Customer-level data rights and vendor authorization - Private interface / export specification from PharmaForce - Non-prod or test tenant / sample files - Clear system-of-record decision for Beacon submission - Claim-level identifiers preserved across PharmaForce and Beacon	Fallback / design decision Use secure structured export/EDI while a richer API path is being enabled. The common Shields adapter means the downstream model does not change. Recommendation: Open technical diligence by requesting PharmaForce's customer interface pack. Do not equate "API-driven Beacon" with "public customer API."
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Public evidence reviewed: [340B Solutions / ClaimsConnect](#) | [Rebate program / API claim](#) | [Split Billing / Multi-EDI](#)

The Craneware Group

Sentinel / Sentrex / Trisus | Proven scheduled SFTP, plus internal rebate workflow integration



FEASIBILITY High for SFTP; Medium for external API	BUILD EFFORT 2-4 engineer-weeks	ELAPSED TO PRODUCTION 4-6 weeks	RISK CASE 6-8 weeks if additional custom report feeds are required
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What is proven publicly

- Craneware publicly documents scheduled SFTP delivery of Claims, Ordering Problems, Unreplenished Costs, Audit, Bulk Dispensations and other reports for automatic integration with hospital systems.
- Craneware documents Trisus rebate functionality integrated with Sentinel and Sentrex data for automated rebate submission and reconciliation.
- The platform family already processes 340B, claim, compliance and medication-financial data at enterprise health systems.

What still requires vendor / customer confirmation

- No public external customer API specification was found for Sentinel/Sentrex/Trisus 340B data; commit to the SFTP baseline unless a private API is supplied.
- Confirm which modules each Shields health-system partner uses; the available data products differ between Sentinel, Sentrex and Trisus.
- Validate whether standard report exports include the exact claim-level source keys, qualification results, rebate status and reversal history required by the Shields canonical model.

Recommended connector design

Recommended technical path



Scheduled SFTP exports are publicly documented; Craneware also documents Trisus-integrated rebate workflows, but no public external customer API was found.

Direction	Inbound claim/qualification/audit and financial-status data from Sentinel/Sentrex/Trisus to Shields.
Transport	Scheduled SFTP is the proven direct path; use file-level controls, file schema versioning and daily ingestion.
Mapping	Preserve product/module source, claim keys, NDC, site / contract pharmacy, match/eligibility status, audit and replenishment context.
Beacon relationship	Craneware can operate its own Trisus rebate workflow; Shields should still separate TPA qualification from Beacon financial status in the reconciliation model.
Upgrade path	If Craneware provides a supported API/data service, replace only the transport adapter; canonical and ledger layers remain unchanged.

Dependencies / acceptance conditions

Required to hold the estimate • Module footprint confirmed per health system • SFTP exports enabled and sample report dictionary available • Required report/fields can be scheduled daily • Historical/backfill approach agreed • If using Trisus rebate status, define how Beacon IDs and payment references are exposed	Fallback / design decision Use scheduled SFTP directly into the Shields file gateway. If required claim-level fields are absent, add one vendor-curated export rather than screen-scraping or manual report automation. Recommendation: Use Craneware as the second file-based pattern after Verity; both can share the same secure-file connector framework.
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Public evidence reviewed: [SFTP scheduled reports](#) | [Rebate integration](#) | [340B portfolio](#)

Macro Helix: 340B Architect

Priority TPA | Strong integration capability, but generic direct API is not publicly documented



FEASIBILITY Medium	BUILD EFFORT 3-5 engineer-weeks	ELAPSED TO PRODUCTION 5-7 weeks	RISK CASE 7-9 weeks if a custom export must be commissioned
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What is proven publicly

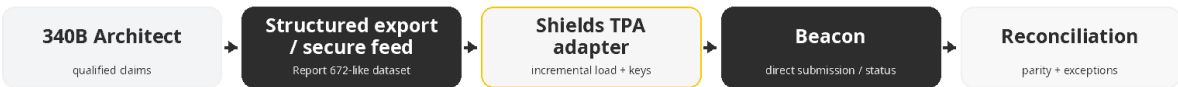
- Macro Helix publicly instructs customers to run Report 672 to display 340B-qualified claims for manufacturer data-sharing workflows.
- McKesson publicly documents a certified EnterpriseRx-to-Macro Helix interface using X12/EDI 850 purchase orders and 855 acknowledgements, proving the product can support structured external integration.
- 340B Architect is designed around automated qualification, replenishment and audit trails across hospital / contract-pharmacy environments.

What still requires vendor / customer confirmation

- The certified EnterpriseRx interface is a specific integration and should not be interpreted as a generic customer API for arbitrary 340B Architect data extraction.
- No public general REST API specification was found; obtain Macro Helix’s customer data-export / integration options and confirm whether Report 672-like data can be scheduled machine-to-machine.
- Confirm source keys, reversal/requalification representation, historical data and whether the feed contains enough fields to link each qualified claim to Beacon and cash.

Recommended connector design

Recommended technical path



Public evidence proves external EDI interfaces and qualified-claim reporting; a generic customer REST API for 340B Architect is not publicly documented.

Direction	Inbound 340B-qualified claim / dispense data and TPA source identifiers from 340B Architect.
Preferred transport	Customer-authorized structured export / secure feed; API if Macro Helix provides a supported private integration service.
Pragmatic baseline	Automate a Report 672-like qualified-claim extract through supported vendor mechanisms rather than relying on manual report pulls.
Existing interface evidence	Treat 850/855 EnterpriseRx connectivity as proof of interface maturity, not as the specific Shields interface to be built.
Beacon relationship	Use qualified claim data as input to a separate Shields Beacon adapter; preserve Macro Helix qualification evidence for audit.

Dependencies / acceptance conditions

Required to hold the estimate - Macro Helix customer support / implementation owner assigned - Supported machine-to-machine export mechanism confirmed - Data dictionary + representative qualified / reversed / rejected samples - Historical backfill availability - Health-system / pharmacy IDs and 340B IDs available for crosswalk	Fallback / design decision Begin with a vendor-supported scheduled flat-file/SFTP export. Manual Report 672 output is acceptable only for feasibility testing, not as the target operating model. Recommendation: Classify Macro Helix as an interface-confirmation dependency in Week 1; do not commit to REST until the private data-exchange package is received.
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Public evidence reviewed: [Macro Helix data sharing / Report 672](#) | [EnterpriseRx interface evidence](#) | [McKesson pharmacy technology](#)

Pillr Health / RxStrategies

Priority TPA | Beacon-ready ecosystem with platform-lineage dependency



FEASIBILITY
Medium

BUILD EFFORT
3-5 engineer-weeks

ELAPSED TO PRODUCTION
5-7 weeks

RISK CASE
7-9 weeks if platform lineage or interface ownership is unresolved

What is proven publicly

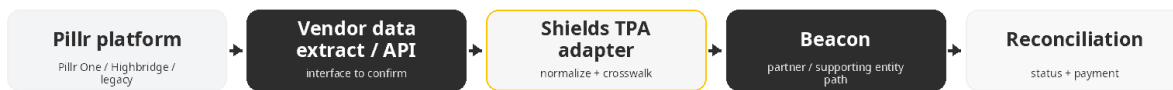
- Pillr Health states that it is registered as a supporting entity within Beacon and assists clients with setup, NDC mapping, claim routing and system readiness for rebate submissions.
- Pillr One advertises visibility across claims, accumulations and purchasing with data connections across EHR, eRx and wholesaler activity.
- Pillr also supports financial reconciliation through Highbridge and is registered with CMS as an MTF Third-Party Support Entity; it completed NCPDP setup as a Pharmacy Remit, Reconciliation and Payment Vendor.

What still requires vendor / customer confirmation

- No public external customer API/SFTP specification was found that is sufficient to design the Shields adapter without vendor input.
- Pillr was formed from RxStrategies, Hudson Headwaters 340B and RPH Innovations; first confirm which current or legacy product stack each Shields health-system relationship actually uses.
- Confirm whether Pillr expects to remain the Beacon supporting entity/submission owner or will expose qualified claims so Shields can submit directly.

Recommended connector design

Recommended technical path



Pillr documents Beacon supporting-entity readiness and multi-source data connectivity; the exact external interface depends on the deployed Pillr / RxStrategies product footprint.

Direction	Inbound claim / qualification / accumulation / purchasing and rebate-related status as applicable to the deployed Pillr product.
Preferred transport	Vendor-supported data API / managed machine-readable extract after product footprint is confirmed.
Fallback	Secure file or report-level extract into the common Shields 340B adapter while API enablement is evaluated.
Crosswalk	Map legacy RxStrategies / Pillr One / Highbridge identifiers to the same Shields claim, 340B ID, site and pharmacy master data.
Beacon relationship	Decide per covered entity whether Pillr remains supporting entity or Shields is separately authorized as partner; avoid duplicate submission paths.

Dependencies / acceptance conditions

Required to hold the estimate

- Deployed Pillr/RxStrategies product lineage confirmed
- Data export/API entitlement and spec received
- Beacon supporting-entity ownership agreed
- Representative data across claims / reversals / exceptions
- Migration/change roadmap understood so connector is not built against a retiring surface

Fallback / design decision

Use a vendor-managed secure export as the first production path if no customer API is offered. Keep the adapter contract independent of the underlying Pillr product so future migrations do not change the reconciliation layer.

Recommendation: Treat product-footprint confirmation as the first technical task; it is the main determinant of interface availability and estimate confidence.

Public evidence reviewed: [Rebate readiness / Beacon](#) | [Pillr One](#) | [CMS TPSE / NCPDP setup](#) | [Pillr formation](#)

Adjacent Connectivity Required for the Full Financial Control Loop

StatusNeo

Direct TPA / Beacon connectivity solves the 340B dependency, but claim-to-cash reconciliation still needs several adjacent systems

The target architecture should not force all sources into the same integration method. The connector fabric should support the best interface for each source while preserving one canonical reconciliation model.

Component	Publicly proven / expected interface	Role in the platform	Recommendation / key dependency
Inmar	Public Inmar healthcare Swagger exists with reconciliation-oriented endpoints; exact Shields dataset / entitlement not proven	Existing pharmacy reconciliation / AR / exception source during coexistence	Validate contract-specific API or feed. Keep as comparison / control source while direct TPA + Beacon connectors prove parity; rationalize later.
CMS / Medicare Transaction Facilitator	CMS publishes MTF DM/PM technical guidance; manufacturer file exchange uses AWS S3; MTF supports refund/payment data and X12 835 for dispensing entities	MFP / IRA-related refund and payment status where applicable	Confirm Shields / customer enrollment and which MTF data path is available to the dispensing entity or authorized third-party support entity.
Epic / Clearinghouse / Payer	Epic publicly supports X12 837, 999, 277CA, 276/277 and incoming 835 interfaces with major clearinghouses and banks	Medical-benefit claim status, denial / underpayment and remittance workflow	Reuse the health system's existing Epic / clearinghouse workflow. Do not build a second denial platform; route status / exceptions and consume outcome.
PBM / PSAO / Switch	Pharmacy adjudication and settlement commonly use NCPDP transaction standards plus payer/switch settlement files	Traditional pharmacy-benefit reimbursement / adjustments / reversals	Identify the authoritative claim/adjudication feed and preserve the full transaction chain, not just the final paid record.
Bank systems	Bank API / SFTP / BAI2 / ISO 20022 / ACH-related files vary by bank	Settled cash and trace reference: final proof of receipt	Obtain cash feed from each bank; map trace/reference to remittance/rebate. Do not close a claim solely because a remittance exists.
ERP / GL	Vendor-specific API / journal / file integration	Optional downstream financial-close / accounting output	Read/report first. Add controlled journal write-back only if finance requires it after reconciliation rules stabilize.

Design implication

Direct-source does not mean API-only

The common connector fabric should explicitly support four production patterns: (1) API/SDK for Beacon and any TPA that exposes a supported API, (2) SFTP/structured files for Verity/Craneware and vendor exports, (3) healthcare EDI/X12/NCPDP for claim/remittance ecosystems, and (4) banking/ERP interfaces for cash and financial close. The canonical data and reconciliation layers remain independent of transport.

Additional access items to validate once health systems are selected

- Health-system-specific clearinghouse / payer topology (e.g., Availity, Waystar, Change/Optum) and whether denial status should route to Epic Resolute or another RCM work queue.
- Bank account / feed ownership, remittance-to-cash matching references and whether payments are aggregated across claims or covered entities.
- PBM/PSAO/switch settlement and reversal data availability, including NCPDP claim identifiers and rebill chains.
- Whether ERP/GL write-back is in MVP or a later financial-close enhancement.

Public evidence reviewed: [Epic Claims & Remittance](#) | [CMS MTF](#) | [Inmar RCM](#) | [Inmar Swagger](#)

Feasibility Gate & Integrated Delivery Timeline

StatusNeo

How to convert public evidence into a commit-ready connector plan before the build is locked

Recommendation to McKinsey / Shields

Keep the 26-week platform plan, but use Weeks 1-3 as an explicit connectivity gate: obtain each vendor's private interface pack / credentials, prove representative golden claims through the strongest supported interface, and re-baseline only connectors requiring vendor custom development.

Connector workstream: realistic calendar view with 3 integration engineers

Timing	Focus	Activities / outcome
Weeks 1-2	Feasibility & access	Open partner/vendor onboarding in parallel; receive Beacon tokens/SDK pack; Verity/Craneware SFTP enablement; request PharmaForce/Macro/Pillr private interface packs; confirm TPA footprint and data rights.
Weeks 2-4	Connector framework	Build common API/SDK gateway, SFTP/file gateway, secrets, schema registry, source contracts, idempotency, retries/replay, audit and monitoring. Beacon adapter skeleton begins.
Weeks 3-6	Wave 1	Beacon + Verity + Craneware working connections in lower environment; load representative data; map golden claims; validate source counts and status semantics.
Weeks 5-9	Wave 2	PharmaForce + Macro Helix + Pillr adapters once interface packs are confirmed; use secure file patterns where direct customer API is not available.
Weeks 8-14	Adjacent + hardening	Inmar coexistence, CMS/MTF, banks and selected health-system interfaces; in parallel complete backfill, reversals, partial-payment handling, security, monitoring, runbooks and data-parity checks.
Weeks 15-24	Platform completion	Reconciliation rules, exceptions, reporting views, UAT, production cutover and operating model complete in parallel with final source tuning.
Weeks 25-26	Hypercare	Production stabilization, connector tuning, reconciliation monitoring, defect closure and structured BAU handoff.

Week 3 gate: required evidence before committing the connector baseline

Beacon	Partner tokens / Read-Write permission, SDK/API pack, non-prod access and successful test submission/retrieval.
Verity / Craneware	SFTP credentials, scheduled export definition and representative files with required claim-level matching keys.
PharmaForce / Macro / Pillr	Private interface/export specification, customer entitlement, sample data and named vendor technical owner.
All vendors / security	History/backfill, corrections/reversals, volumes, limits, support/versioning plus BAA/data-use permissions, covered-entity authorization and minimum-necessary PHI controls.

Go / Amber / Red decision rule

GO Supported interface + access path + sample data proven	AMBER Supported path exists but onboarding / field coverage is unresolved	RED No supported machine-to-machine path without vendor custom work
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Commit recommendation

Target direct-source connectivity is feasible, but the commitment should be hybrid rather than API-only. If the Week 1-3 access gate clears, six production-capable priority connectors in ~10-14 weeks is a realistic range inside the 26-week program; if 2+ TPAs require vendor custom development, plan for ~14-18 weeks.

Public evidence reviewed: Final proposal | [Beacon partner docs](#) | [Verity SFTP](#) | [Craneware SFTP](#) | [PharmaForce](#) | [Macro Helix](#) | [Pillr](#)